INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Oral etoposide · Days 1–7 twice daily

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00388
Version 4

> Take your prescribed etoposide capsules twice daily on Days 1–7. Days 8–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

Treatment stages

01 **Chemotherapy course**
Review each cycle

02 **Review and next plan**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Oral etoposide · Days 1–7 twice daily · National Cancer Control Programme (NCCP) 00388

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

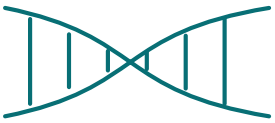
Your chemotherapy

This treatment aims to slow or shrink extensive small-cell lung cancer when intravenous or combination chemotherapy is unsuitable. Benefit varies; your team explains the aim for you.

TOPOISOMERASE-II INHIBITOR CHEMOTHERAPY

Etoposide capsules

Supplied brand may vary



Disrupts deoxyribonucleic acid (the cell's genetic material) (DNA) copying and repair

Etoposide interferes with topoisomerase II, an enzyme involved in DNA structure and repair. DNA damage can stop cancer cells dividing. Healthy blood-forming and other cells can also be affected. Absorption by mouth varies, so oral and intravenous doses are not interchangeable.

Medicine reaches cells → DNA damage persists → Cell division disrupted

What to watch for

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

My treatment

Evidence and limits

Older studies reported tumour responses with oral etoposide. The evidence is limited, and the different oral schedules cannot be assumed to have the same benefit or risks. A small uncontrolled study of a 21-day oral course does not establish a survival advantage or validate all three NCCP schedules. Its participants had recurrent cancer and substantial treatment toxicity; do not transfer its response percentage to an individual.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Oral etoposide · Days 1–7 twice daily · NCCP 00388

Your treatment course

Treatment stages

01 **Chemotherapy course**
Review each cycle

02 **Review and next plan**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

REVIEW EACH CYCLE

Chemotherapy course

Cycle | Review

Take your prescribed etoposide capsules twice daily on Days 1–7. Days 8–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

INDIVIDUAL PLAN

Review and next plan

Review

Your team reviews benefit and side effects and agrees whether to continue, change or stop.

Relative educational plan; your team confirms actual appointments and changes.

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Check	Before treatment	During the cycle	Before next course	After treatment
Safety blood tests	Blood count, kidney and liver profile	Extra tests if needed; oral etoposide regular dates confirmed by team	Blood count, kidney and liver review	Follow abnormal results as advised

Check	Before treatment	During the cycle	Before next course	After treatment
Disease assessment	Confirm treatment aim	Symptoms reviewed	Scans or other tests on consultant plan	Agree next treatment or follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00388 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Oral etoposide · Days 1–7 twice daily: treatment and checks

Extensive small-cell lung cancer (SCLC) · Days 1–7 twice daily

Take your prescribed etoposide capsules twice daily on Days 1–7. Days 8–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment days	Between courses	Course review
Treatment NCCP 00388	Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist.	Take your prescribed etoposide capsules twice daily on Days 1–7. Days 8–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.	Follow support plan and report symptoms	Review benefit/ toxicity; next plan
Safety tests NCCP 00388	Blood count, kidney/liver tests	Review symptoms; extra tests if needed	Blood count, kidney and liver tests before treatment and regularly at team-specified intervals. Scans and follow-up follow the consultant plan.	Follow abnormal results

What each check is for

1 **Before each course**

Before starting

Safety and plan may change

Ask / **act:** Team confirms next treatment

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before treatment

Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist.

STEP 2

Treatment visit or home course

Take etoposide capsules on an empty stomach using the exact prescribed dose and days. Do not open, chew or crush them. Capsule strengths, quantities and storage follow the supplied pharmacy label. Oral and intravenous doses are not interchangeable.

STEP 3

Bring to review

Bring your medicine list, dose record and questions; report new symptoms before starting another course.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Oral etoposide · Days 1–7 twice daily · NCCP 00388

Your 21-day cycle.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
Prescribed capsules	 **Days 1–7** intravenous (IV)
No planned chemotherapy	 **Days 8–21** Blood-count and symptom recovery varies.

Effects vary; no risk curve or guaranteed recovery day is shown.

The next cycle starts 21 days after this one, if your team confirms it.

> Do not use this page to calculate or change doses.

Between-treatment notes

Day 1–7: Treatment days

Take your prescribed etoposide capsules twice daily on Days 1–7. Days 8–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review. Follow the confirmed prescription; report problems promptly.

Day 8–21: Between treatments

No planned chemotherapy; continue prescribed support and monitoring. Do not restart until the team confirms the next cycle.

Support around the medicines

1 Sickness and bowel symptoms

Use your prescribed prevention or rescue medicine. Report persistent vomiting, diarrhoea or inability to drink; fever with severe tummy pain needs urgent assessment.

2 Blood tests and review

Blood count, kidney and liver tests are checked before treatment and regularly on dates set by your team for the selected schedule.

3 **Pregnancy, fertility and medicines**

Discuss fertility and contraception before treatment. Tell the team about pregnancy, breastfeeding, all medicines, supplements and vaccines. Check before changing any medicine.

4 **Missed or vomited capsules**

Do not double a missed dose or take replacement capsules after vomiting without team advice. Record what happened, contact the oncology pharmacist/team, and follow the next scheduled dose in the agreed plan.

5 **Safe handling**

Take etoposide capsules on an empty stomach using the exact prescribed dose and days. Do not open, chew or crush them. Capsule strengths, quantities and storage follow the supplied pharmacy label. Oral and intravenous doses are not interchangeable. Keep away from children; ask the pharmacy how to return unused or damaged capsules.

Between treatments

Keep your prescription and contact plan to hand.

Blood tests and review

Pregnancy, fertility and medicines

When to call

Blood counts

An infection can be serious even after treatment days have finished. A normal earlier blood test does not rule out a new problem.

Tests and review

Blood count, kidney and liver tests before treatment and regularly at team-specified intervals. Scans and follow-up follow the consultant plan.

Scans and tumour markers

Your consultant decides which are relevant and when. Do not assume a fixed imaging interval or that tumour markers are required for everyone.

After treatment

Agree follow-up, symptom support and the next plan before the course ends.

Questions worth asking

Why can treatment be delayed?

Blood results or symptoms may make a delay or dose change safer. Your team confirms when to restart.

Are capsules gentler than an infusion?

No. Oral chemotherapy can cause serious toxicity and needs blood tests and review. Oral and intravenous doses cannot be exchanged.

When will I have scans?

Your consultant sets the timing for your disease and treatment plan. No universal scan or tumour-marker interval is specified here.

> **Try explaining the plan back:** Explain which days you take treatment, when your next blood tests are due and whom you will call tonight if unwell.

Check your exact treatment This guide covers only Days 1–7 twice daily. Do not combine it with another etoposide schedule.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Oral etoposide Days 1–7 twice daily support

Active medicines: etoposide

Emetogenic risk: low

Medicine	Dose / route	When
Individually prescribed sickness medicine	As prescribed under oral low-risk guidance	As required; persistent symptoms need review No duplicated preventive courses. Product interactions and patient factors apply.

Additional phase-specific support

- Follow a written prescription with start/stop days.
- No routine premedication required; local policy applies.
- Review diarrhoea, hydration and capsule handling.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hair / energy

Etoposide

Hair loss and tiredness may occur; report marked weakness or dizziness.

3 Chest / breathing

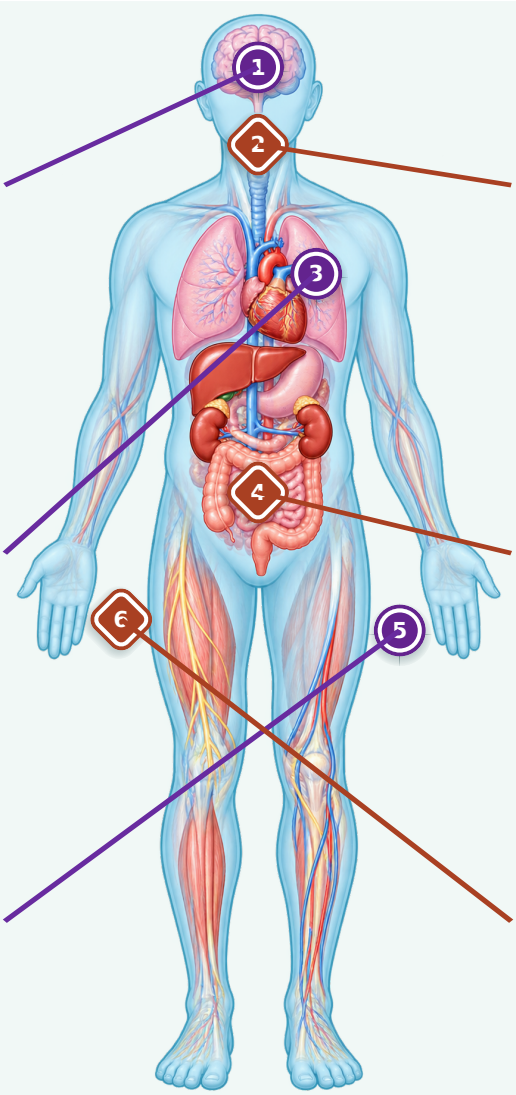
Etoposide

New cough, breathlessness or chest pain needs urgent assessment.

5 Skin / allergy

Etoposide

Report spreading rash; swelling or breathing difficulty needs immediate help.



2 Mouth

Etoposide

Tell the team about ulcers or painful swallowing; ask for help if you cannot drink.

4 Stomach / bowel

Etoposide

Report persistent diarrhoea or vomiting. Fever with severe tummy pain is urgent.

6 Blood counts

Etoposide

Fever, unusual bruising or bleeding needs an immediate call. The marker is schematic.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Your record and contact card

Oral etoposide · Days 1–7 twice daily · National Cancer Control Programme (NCCP) 00388

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record the actual cycle and any omitted, delayed or changed doses with your team. A completed stamp does not authorise another dose.

Etoposide capsules · Topoisomerase-II inhibitor chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Bleeding / bruising	□ None	□ New small bruise / brief bleed: report	□ Repeated bleeding: call today	□ Bleeding that will not stop: urgent help	□ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Extensive SCLC · Days 1–7 twice daily · NCCP 00388 v4 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Oral etoposide

Days 1–7 twice daily; 21-day cycle

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.32 · NCCP 00388 v4 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

Sources and further information

Oral etoposide · Days 1–7 twice daily · National Cancer Control Programme (NCCP) 00388

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00388 v4 · pNCCP 00388 supportive care; NCCP antiemetic guidance**
   https://healthservice.hse.ie/documents/6911/388_oral_etoposide_.pdf
2. **HPRA Vepesid product information**
   https://assets.hpra.ie/products/Human/30580/Licence_PA2239-006-001_27022024163103.pdf
3. **NCCP NCIS medical oncology antiemetic guidance V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
4. **Johnson 1990 oral etoposide phase II study**
   https://pubmed.ncbi.nlm.nih.gov/2170589/